

CUTANEOUS

LYMPHOID

HYPERPLASIA

Epidemiology

Cutaneous lymphoid hyperplasia (CLH) has a worldwide distribution and affects all races and ethnic groups. It occurs in both adults and children. Females are more commonly affected than males.

Etiology and Pathogenesis

CLH is characterized by a relatively dense lymphoid infiltrate, centered in the reticular dermis, that is usually B-cell rich and may resemble lymphoma clinically and/or histopathologically. Many terms are used to refer to this type of pseudolymphoma, including Spiegler-Fendt sarcoid, lymphocytoma cutis, lymphadenosis benigna cutis, and cutaneous lymphoplasia. Cutaneous lymphoid hyperplasia is the preferred term because it accurately describes the underlying pathophysiology of the lesion and is unlikely to be confused with terms used to describe various forms of cutaneous lymphoma.

In most cases, CLH is idiopathic; however, some lesions are associated with exposure to foreign antigens from arthropods (bites, stings, infestations), infections (herpes zoster, *Borrelia burgdorferi*), tattoos, acupuncture, trauma, gold jewelry, vaccinations, hyposensitization injections, or medications. Medications that may induce CLH include phenytoin, carbamazepine, phenobarbital, β -blockers, calcium channel blockers, angiotensin-converting enzyme inhibitors, allopurinol, D-penicillamine, penicillin, mexiletine chloride, cyclosporine, and agents that inhibit binding of histamine to H₁, H₂, or H_{1c} receptors. These drugs include conventional H₁ and H₂ antagonists, as well as tricyclic and nontricyclic antidepressants and phenothiazines. It has been proposed that, rather than being the target of an immune response themselves,

these latter agents may alter lymphocyte reactivity in a way that promotes a CLH response to other antigens in some individuals.

CUTANEOUS PSEUDOLYMPHOMAS

AT A GLANCE

A diverse group of skin disorders characterized by dense lymphoid infiltrates mimicking various types of cutaneous lymphomas.

Two major immunologic categories:

- (1) Mixed B- and T-cell (cutaneous lymphoid hyperplasia, Kimura disease, angiolymphoid hyperplasia with eosinophilia, Castleman disease)
- (2) T-cell (pseudo-mycosis fungoides, lymphomatoid contact dermatitis, Jessner's lymphocytic infiltration of the skin)

Most are idiopathic, but some are reactions to foreign antigens introduced locally or systemically.

Usually present as violaceous, solitary, or localized papules, plaques, or nodules. Some cases harbor occult dominant lymphoid clones; occasional cases progress to overt cutaneous lymphomas.

Clinical Findings

APPROACH TO THE PATIENT

Although the details of the workup for individual patients will vary with the specific type of pseudolymphoma being considered, certain data are generally important for establishing the correct diagnosis, identifying potential causative factors, and ruling out an underlying lymphoma. The clinical history should elicit information about the duration and symptomatology of lesions; the nature and pace of clinical progression; past treatments; local or systemic exposure to

foreign antigens, including medications; and personal or family history of other lymphoproliferative disorders.